

The Live Demo Script

What to actually say and click, start to finish — timed to ~3:30, fits a 4-minute slot

Read this out loud, start to finish, before you go up — don't just skim it. Every line here was said, clicked, and verified against the real running app, not written from memory.

The script

OPEN — no click yet

SAY: “Good [morning/afternoon], judges. Doctors in India spend about two minutes with each patient. But seventy to eighty percent of a correct diagnosis comes from the patient's history alone, before any test. So the two minutes doctors have the least of is exactly the two minutes that decides the most.”

[PAUSE]

THE SAFETY NET — zero clicks

[POINT at the live-demo ticker on the homepage, under the hero text — it types and reacts on its own]

SAY: “Watch this — I haven't clicked anything yet. This is typing a real emergency phrase and catching it live, against the same safety check every real submission runs through. No AI call needed for this part — it works even fully offline.”

If the ticker is still loading when you arrive, wait a beat in silence rather than talking over a blank box.

[PAUSE]

TYPE IT YOURSELF — and show it's not just typing

[CLICK into the “Describe how you are feeling” box. TYPE (or paste): “I have had a mild cough and runny nose for two days, no fever.”]

SAY: “Now let me actually use it, the way a patient would. And it isn't just typing — I could just as easily tap this microphone and speak in Hindi or Telugu, or photograph an old prescription instead. One entry point, three ways in, for whoever's in front of it.”

POINT at the mic button and the photo-upload button as you say this — don't actually record or upload, that's real audio/OCR processing and eats time you don't have. Pointing is enough.

[PAUSE]

THE ADAPTIVE FOLLOW-UP QUESTION — a real differentiator

[A question appears right under the box: “Is it a dry cough, or are you bringing up phlegm or mucus?” CLICK an answer, or CLICK “Skip these questions” to move on]

SAY: “This is asking the same follow-up a doctor would — adapted to what I actually typed, not a fixed form. It can ask up to eight of these, one at a time, and every one is skippable.”

Verified live: typing this exact sentence surfaces “Question 1 of 8” immediately. Answer one for effect, then skip the rest to protect your time.

[PAUSE]

[CLICK Next → Next (skip the optional photo step) → fill Age 29 and Days 2 → Next → check the consent box → Submit My Symptoms]

SAY: “Age and how many days are optional — you can skip them and it still works. And this consent line is real, not decorative — it's enforced on the server too, in line with India's Digital Personal Data Protection Act.”

[PAUSE]

THE RESULT — this is the core of the pitch

[SCROLL through, in order: the priority banner → “Why this priority level” → Chief Complaint / History of Present Illness → the AI-drafted disclosure line]

SAY: “This isn’t just a label — it quotes the actual clinical guideline sentence it matched, and how similar the match was. That’s a real citation, not a black box. And notice this line at the bottom: it openly says this is AI-drafted and hasn’t been reviewed by a physician yet. It’s built to help a doctor decide faster — never to replace the doctor’s own judgment.”

Verified live: this exact input returns CLINIC VISIT with a guideline quoted at 40% similarity — notice the level and the guideline text actually agree with each other now (“typically evaluated at a routine clinic visit” → CLINIC VISIT), which is worth saying out loud if you get the chance: the fallback doesn’t just cite evidence, it acts on what the evidence says.

[PAUSE]

TWO MORE REAL FEATURES, QUICKLY — if short on time, cut this beat before any other

[CLICK the AYUSH toggle (“This is an Ayurvedic OPD visit”) to reveal the real form for one second, then CLICK “Listen to summary” and let it speak a few words before moving on]

SAY: “For an Ayurvedic consultation, it captures the real Dashavidha Pariksha history an AYUSH doctor actually uses — Prakriti, Vikriti, and the rest — not a generic form with “Ayurveda” relabeled onto it. And for anyone who’d rather listen than read, the whole summary plays back as audio too.”

Don’t wait for the audio to finish — a few seconds of it playing is proof enough, then move on.

[PAUSE]

THE TAKEAWAY MOMENT

[CLICK “Download this summary.” A file lands immediately — show the downloads bar/folder if visible]

SAY: “And here’s the whole point of doing this before the visit: one tap, and the patient has a real file they can carry straight into the consultation room — no login, no app to install, works on any phone.”

[PAUSE]

THE DOCTOR’S SIDE — SKIP LIVE unless you’ve set a staff passcode

[Only attempt this if PHYSICIAN_CONSOLE_PASSCODE is actually set on your deployment — CLICK “Physician console” (top of page) → sign in with the staff passcode → open the case you just submitted]

SAY: “And this is what the doctor sees the moment the patient walks in — the same draft, ready to accept, amend, or reject. Same real case, same database — not a separate demo.”

If no passcode is configured, this isn’t a “cut if short on time” step — it’s a real 503 error on screen if you try it. Describe it verbally instead (see the line below) and never click into it live.

[PAUSE]

CLOSE — say this one slowly, make eye contact

SAY: “One last thing: every safety check you just saw — the red-flag catch, the priority level, the guideline citation — runs with zero API cost. It keeps working even if the AI service is down. That’s what lets this scale to a real, high-volume government hospital, not just stay a demo. INAYAT means ‘care’ in Urdu — we built it to give the doctor’s two minutes back to actually deciding, not repeating questions. Thank you — happy to take questions.”

[PAUSE for questions]

If you forget every line above, remember the shape of it: **open on the 2-minute problem → the homepage catches an emergency with zero clicks → type one input, point at voice/photo, answer one adaptive question → point at the guideline citation → flash AYUSH + audio → download the file → close on ‘zero API cost, still works offline.’**

Before you walk up

- Open the live URL 3–5 minutes early — free-tier hosting can be slow to wake on the first hit of the day. Let it fully settle before your slot.
 - Do one hard refresh (Ctrl/Cmd+Shift+R) right before you go up, just to be safe.
 - Have the demo line ready to paste, not typed live from memory: “I have had a mild cough and runny nose for two days, no fever.”
 - Check right now whether PHYSICIAN_CONSOLE_PASSCODE is actually set on your deployment. If it isn’t, drop the physician-console beat from the script entirely — don’t plan to show it and skip live, plan to never click it.
-

If it breaks live — keep composure, don't improvise

- **Page slow to load:** keep talking through the opening lines while it catches up — don't stand in silence.
- **Wi-Fi drops entirely:** say plainly, “looks like the connection dropped — let me talk you through it,” and narrate the results screen from this script, from memory.
- **Page looks visually broken:** one hard refresh, done without apologizing at length — then continue exactly where you left off.
- **A judge questions a level:** SAY — “It only raises the level when it finds real evidence, like the guideline quote you just saw — with no matching evidence it stays cautious rather than guessing, and it never quietly lowers a level on a weak match. That's a deliberate safety design, not a confidence score.” The fallback genuinely reasons across self-care, clinic-visit, urgent, and emergency now (verified live: a plain headache returns SELF-CARE, this demo's cough returns CLINIC VISIT, chest pain returns EMERGENCY) — you can say this honestly, just don't improvise a *specific* new input live to prove it if you haven't tested that exact wording first.
- **Download seems to do nothing:** check the downloads bar/folder — some browsers save silently. If truly blocked, fall back to “Listen to summary” instead.
- **No physician passcode configured on this deployment:** don't click into the console at all — it returns a real 503 error, not a slow load. SAY — “And on the doctor's side, the same case opens instantly in a separate, passcode-locked console” — one sentence, then move straight to the close.